

Tue, Jan 13 at 11:46 PM I work

as an orthotist in Sydney, and one of my patients is about to begin a 2-month Bioness community trial. Her condition is MS. She wants to purchase BIONESS through NDIS

funding after using it for two months. What would be the best way to conduct the usage program over the next two months?

That is a good question.

The key to a 2-month Bioness L300 (or Go) community trial is structuring it to simultaneously generate “safety ÿ efficacy ÿ functional change ÿ NDIS evidence.”

Below is an 8-week program that can be

immediately applied from an orthotist's perspective.

1ÿÿ Overall Trial Goal

The goal is to clearly prove the following four things over a period of two months.

Improvement of pedestrian safety

Reduced risk of toe drag, stumble, and fall;

improved functional

mobility; speed, endurance, and community

ambulation; reduced fatigue and

compensation patterns; hip hike, circumduction, and overuse pain

Functional Benefits Compared

to AFOs Securing Evidence for Why Bioness is

'Necessary' 2ÿÿ Pre-start (Week 0) – Baseline Setting

Clinical Evaluation

Muscle strength: DF / PF / EV / IV

Tone / spasticity (MAS)

Sensation

ROM (ÿÿ ankle DF with knee ext/flex)

Fatigue level (VAS or MFIS)

Functional Evaluation (Required for NDIS Report)

If possible, leave it as a number

10MWT (comfortable + fast)

END

6MWT or 2MWT

FGA or Berg Balance (if possible)

Fall history (last 3 months)

Current walking strategy record

Whether to use AFO

Toe drag, knee hyperextension, hip hike, circumduction. Indoor/outdoor

walking distance: 3ÿÿ. 2-month

Trial Program Structure

Phase 1 – Adaptation Period (Week 1–2)

Goal: Safe wear, stimulus adaptation, creating basic patterns

How to use: 30–

60 minutes per day, primarily walking on flat indoor surfaces.

Usage situations:

Movement within

the house, short walks (5–10 minutes).

Heel switch sensitivity adjustment

Optimization monitoring of gait phase

timing, skin

response,

excessive fatigue, and

balance changes

Phase 2 – Feature Expander (Week 3–4)

Goal: Apply community ambulation

How to use: 1–

2 hours per day in

outdoor environments,

including sidewalks, ramps, and stairs; before

and after; visiting shopping centers, cafes, hospitals, etc.

Interim evaluation (Week

4) recording in which situations Bioness is more effective compared

to AFO

10MWT, TUG re-measurement,

patient-reported

indices, change in “feeling like falling,”

fatigue,

gait confidence

Phase 3 – Real-world Validator (Week 5–6)

Goal: Create a stage where “it is inconvenient without this.”

How to Use: 2–4

hours per day (within the range of continuous wearability). Apply during

the most strenuous situations: Grocery

shopping,

public

transportation, commuting

to work/school, fatigue after long

walks. Comparison Points

AFO vs. Bioness crossover week: Have patients

try both in the same environment and encourage

them to describe the differences themselves.

Phase 4 – Evidence Collector (Week 7–8)

Goal: Complete data for NDIS submission

Final evaluation

10MWT / TUG / 6MWT Re-measurement

Whether to fall / near falls

Fatigue change

Changes in community participation,

frequency and

distance

of outings, confidence 4ÿÿ “Usage Diary” to request from the patient

It serves as extremely strong data for NDIS grounds.
Machine Translated by Google

Brief daily record Usage

time Location

of use (Home / Outdoors / Shopping center, etc.)

Feeling Sense of stability ÿ / Fatigue

ÿ / Pain ÿ Problems

Example: "After wearing Bioness, my feet didn't drag at the supermarket, so I felt stable while pushing the shopping cart. Previously, even when wearing AFOs, I experienced severe fatigue after 10 minutes."

5ÿÿ Collaboration Points with Physio / OT

Physio

Gait training with FES

Step length symmetry

Endurance building

OT

Recording changes in daily activity

performance linked to community

access goals (shopping, public transport, work participation)

NDIS responds to 'function' and 'participation'.

Emphasize changes in daily activities rather than simple muscle strength improvement.

6ÿÿ Messages You Must Create in NDIS Reports

You should be able to say this in the report two months from now:

"Client demonstrated clinically significant improvement in gait safety, endurance and community participation with Bioness compared to conventional orthotic management.

Without FES, she remains at high risk of falls and activity restriction."

Key Comparison

Structure Items AFO Bioness

Significant reduction in sustained toe drag

Fall risk medium–high low

Endurance 10–15 minutes 30–40 minutes

Community access restriction independent

Reduction of High Fatigue

7ÿÿ Clinical Logic to Emphasize as an Orthotist

MS is a variable disease ÿ Adaptive

Neuromuscular Stimulation (FES) is more advantageous for maintaining function than fixed AFOs.

Bioness is not merely assistance, but

neuromuscular retraining + facilitating functional

gait. In the long

term: fall

prevention and reduction of secondary injuries.

Reduced reliance on Physio/support personnel ÿ NDIS cost reduction logic

8-week trial

plan PDF template if you want

Sentence Template for NDIS AT Report

Which materials do you need first?

That is a good point — this stage logbook serves as very strong evidence in NDIS. The core purpose is to **“prove functional degradation in the absence of bioness . ”**

Below is the 3-week (withdrawal period) logbook template after returning Bioness.
It was designed to make the comparison with vs. without clear in the AT Report.

POST-BIONESS WITHDRAWAL LOGBOOK (3-WEEK)
(After Bioness Trial – Patient Record for NDIS Evidence)

Participant Information

Participant Name: __
NDIS Number: __

Trial Completed Date: / __ ' __ / __
Logbook Period: __ __ ' __ to __ ' __ / __

Prescribing Orthotist: ____
 Purpose of This Logbook

You have completed your Bioness trial.

This logbook is to record how your walking feels WITHOUT Bioness, to help your clinical team understand the difference and support your NDIS funding application.

DAILY RECORD (WITHOUT BIONESS)

Date: ____

Mobility Today

Where did you walk today?

- ÿ Home only
- ÿ Short outdoor walk
- ÿ Community (shopping, appointments, etc.)

What activities did you do?

Walking Ability (WITHOUT Bioness)

Walking felt:

- ÿ Very difficult
- ÿ Difficult
- ÿ Manageable
- ÿ Easy

Stability & Safety

Did you feel unstable when walking?
ÿ Yes ÿ No

Compared to when using Bioness, your walking feels:
ÿ Much worse
ÿ Worse
ÿ Same

Fatigue

Much more fatigue ÿ More

fatigue ÿ Same

fatigue

Did you experience:

ÿ Toe drag ÿ

Tripping ÿ

Near fall

ÿ Fall

ÿ None

If yes, please describe:

Activity Limitation

Did you avoid or stop any activity today due to walking difficulty? ÿ Yes ÿ No

If yes, what did you avoid?

Comments

WEEKLY SUMMARY (End of each week)

Week Number: _____

Compared to when using Bioness:

Walking safety: ÿ

Much worse ÿ Worse ÿ Same

Walking endurance: ÿ

Much worse ÿ Worse ÿ Same

Fatigue:

ÿ Much higher ÿ Higher ÿ Same

Activities that are now more difficult WITHOUT Bioness:

ÿ Walking long distances ÿ

Shopping ÿ

Appointments ÿ

Outdoor walking ÿ

Public transport ÿ

Social activities

ÿ Work / study

Details:

Falls / Near Falls This Week:

ÿ None

ÿ Near-falls

ÿ Falls

Details:

Machine Translated by Google

- 1. How has your walking changed since returning the Bioness device?
- 2. What activities are harder or no longer possible WITHOUT Bioness?
- 3. Do you feel less safe walking without Bioness?

ÿ Yes ÿ No ÿ Sometimes

Explain:

- 4. Do you feel you NEED Bioness for safe daily mobility?

ÿ Yes ÿ No

Why:

- 5. If Bioness is not provided, how will this impact your independence?

Clinician Notes (Optional) Key
Strategy of This Logbook (NDIS Perspective)

This document is not a mere record:

ÿÿ Functional decline evidence ÿÿ Risk
increase (falls, fatigue) ÿÿ Participation
restriction ÿÿ “Not a convenience, but
necessity” ÿÿ

in other words,

Trial (improvement) + Withdrawal (deterioration)
= It is a structure that significantly increases the probability of NDIS approval.

If you wish, proceed directly to the

next step ÿ Create this logbook as a Word + PDF file

ÿ
Trial Logbook + Withdrawal Logbook + AT Report Automatic Connection Package ÿ

Planner: A complete AT report including persuasive sentences

I will make it for you all at once.

Would you like me to send it to you as a file?

Tue, May 5 at 8:59 PM Please
give me the file

Completed ÿ We have
created the requested 3-week Withdrawal Logbook after returning the Bioness for you.

Post-Bioness Withdrawal Logbook (3 Weeks)
Word (for editing)

Post_Bioness_Withdrawal_Logbook.docx

PDF (for patient distribution)

Post_Bioness_Withdrawal_Logbook.pdf

This is not just a simple record:

• Evidence of functional decline • Grounds for increased risk of falls •

Participation restriction • Crucial evidence that “Bioness = Need”

Improvement in Trial + Deterioration in Withdrawal = The most powerful combination for persuading a Planner.

If you wish, proceed to the next step

• Trial + Withdrawal + Outcome measure • Automatic AT Report Generation Template •

Final Report for Persuading NDIS Planner (Almost Approval Level)

I will make it for you.

Make it for me

Completed • We have created the requested Trial + Withdrawal data-based automatic connection AT Report template.

NDIS AT Report (Auto-Generated Template) • Word (•••)

NDIS_AT_Report_Auto_Template.docx

PDF (for submission)

NDIS_AT_Report_Auto_Template.pdf

Key Strengths of This Template

This isn't just a template

It is the “ultimate weapon” **that binds all the materials created so far into one** .

Automatic connection structure:

Trial Logbook • Basis for Improvement
Physio Outcome Measures • Objective figures
Withdrawal Logbook • Evidence of degradation

These 3 are combined:

This is not a convenience, but an essential piece of equipment.

It naturally forces the conclusion that...

A structure that actually gets approved well

The most persuasive pattern in NDIS:

• Baseline: Problem present •
Trial: Clearly improved •
Withdrawal: Worsen again

If you wish, proceed to the next step:

Create the "Final AT Report" by inputting actual patient data

Enhanced version of the sentence that makes Planner unable to refuse

Full package with automatic connection to Physio results

I will make it for you.

Shall we

move on to the "Final Version for Actual Submission" next?